

Patient Name: MUNNANGI, Jayanth
Date of Birth: 09/10/1996
Patient ID: PID00052404
Study Description: MRI Knee Rt
Study Date: 30/05/2026

MRI Knee Right

CLINICAL INDICATION:

Twisting injury to the right knee approximately 5–6 weeks ago. Immediately following injury, patient experienced significant pain and was unable to walk for approximately 3–4 days. Ultrasound and X-ray were reportedly normal. Persistent symptoms include ongoing swelling, knee instability and episodes of giving way during walking. Concern regarding possible ligamentous injury/internal derangement, including ACL injury.

COMPARISON:

None available.

TECHNIQUE:

Axial, coronal and sagittal MRI sequences of the right knee.

FINDINGS:

The medial and lateral menisci are intact, with no meniscal tear identified.

The anterior cruciate ligament is deficient and discontinuous, with associated anterior translation of the tibia on the femur, consistent with a complete ACL rupture. The posterior cruciate ligament is intact.

There is fluid superficial to the medial collateral ligament, consistent with an MCL sprain. The lateral collateral ligament complex is intact.

The quadriceps and patellar tendons are intact. The extensor mechanism is normal.

There is a small to moderate knee joint effusion and a small popliteal cyst. There is bone marrow oedema in the posterolateral and posteromedial tibial plateaus, the subchondral aspect of the lateral femoral condyle and the medial aspect of the medial femoral condyle, consistent with a pivot-shift pattern of injury.

CONCLUSION:

Complete anterior cruciate ligament rupture with anterior tibial translation and pivot-shift pattern bone marrow oedema. MCL sprain with small to moderate joint effusion and small popliteal cyst. No meniscal tear identified.

Signed by: Dr Koshy Jacob MBBS, MRCP, FRCR, CCPC, ACC

Consultant Radiologist

GMC No. 5196831

DMC Radiology Reporting

